

AZIMAX-250 TABLET

VIAZlxx-0 (MY)_2

DESCRIPTION

Oblong, white film-coated tablet, bevel-edged, shallow convex with break bar on one side and 'HOVID' embossed on the other side.

COMPOSITION

Each film-coated tablet contains Azithromycin 250 mg.

PHARMACODYNAMICS

Azithromycin is an azalide antibiotic, part of the macrolide family of antibacterials. It binds to the 50S ribosomal subunit of the 70S ribosome of susceptible organisms, thereby inhibiting RNA-dependent protein synthesis.

PHARMACOKINETICS

Following oral administration, azithromycin is rapidly absorbed and widely distributed throughout the body; bioavailability is approximately 37%. The time taken to peak plasma levels is 2-3 hours. The plasma terminal elimination half-life closely reflects the tissue depletion half-life of 2 to 4 days. Protein binding varies with concentration: very low to moderate; approximately 7% at 1 µg/mL, to 50% at 0.02 to 0.05 µg/mL.

Azithromycin concentrates intracellularly, resulting in tissue concentrations 10 to 100 times higher than those found in plasma or serum. Azithromycin is highly concentrated in phagocytes and fibroblasts. Biotransformation occurs in liver, in which approximately 35% metabolized by demethylation. Over 50% of the dose is eliminated through biliary excretion as unchanged drug.

INDICATIONS

Azithromycin is indicated for the treatment of the following infections when known or likely to be due to one or more susceptible microorganisms:

- bronchitis
- community-acquired pneumonia
- sinusitis
- pharyngitis/tonsillitis
- otitis media
- skin and soft tissue infections
- uncomplicated genital infections due to Chlamydia trachomatis

CONTRAINDICATIONS

- This medication should not be used when the patient is hypersensitive to azithromycin or other macrolides.
- Risk-benefit should be considered when hepatic function impairment exists because biliary excretion is the major route of elimination.

WARNING AND PRECAUTIONS

- In the event of severe acute hypersensitivity reactions, such as anaphylaxis, severe cutaneous adverse reactions (SCARs) [e.g. Stevens-Johnson Syndrome (SJS), toxic epidermal necrolysis (TEN)], drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP)], Azimax should be discontinued immediately and appropriate treatment should be urgently initiated.
- Since liver is the principal route of elimination for azithromycin, the use of azithromycin should be undertaken with caution in patients with significant hepatic disease.
- In patients receiving ergot derivatives, ergotism has been precipitated by coadministration of some macrolide antibiotics. There are no data concerning the possibility of an interaction between ergot and azithromycin. However, because of the theoretical possibility of ergotism, azithromycin and ergot derivatives should not be coadministered.
- As with any antibiotic preparation, observation for signs of superinfection with non-susceptible organisms, including fungi is recommended.
- In patients with severe renal impairment (GFR<10ml/min) a 33% increase in systemic exposure to azithromycin was observed.

Prolongation of the QT interval

- Prolonged cardiac repolarization and QT interval, imparting a risk of developing cardiac arrhythmia and torsades de pointes, have been seen in treatment with macrolides, including azithromycin. The risk of QT prolongation, which can be fatal, should be considered when weighing the risks and benefits of azithromycin for at-risk groups including:
 - Patients with congenital or documented QT prolongation
 - Patients currently receiving treatment with other active substances known to prolong QT interval, such as antiarrhythmics of Classes IA and III, antipsychotic agents, antidepressants, and fluoroquinolones
 - Patients with electrolyte disturbance, particularly in cases of hypokalemia and hypomagnesemia
 - Patients with clinically relevant bradycardia, cardiac arrhythmia or cardiac insufficiency
 - Elderly patients: elderly patients may be more susceptible to drug-associated effects on the QT interval
- Infantile hypertrophic pyloric stenosis (IHPS) has been reported following the use of azithromycin in infants (treatment up to 42 days of life). Parents and caregivers should be informed to contact their physician if vomiting and/or irritability with feeding occurs.

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PREGNANCY AND LACTATION

Adequate and well-controlled studies in humans have not been done; azithromycin should be used during pregnancy only if adequate alternatives are not available.

It is not known whether azithromycin is excreted in human milk, caution should be exercised when azithromycin is administered to a nursing woman.

MAIN SIDE/ADVERSE EFFECTS

Gastrointestinal disturbances are the most frequent adverse effect but are usually mild. Transient elevations of liver enzyme values have been reported and, rarely, cholestatic jaundice. Rashes, headache and dizziness may occur. Transient alterations in neutrophil counts have been seen in patients receiving azithromycin.

Cardiac Disorders: Palpitations and arrhythmias including ventricular tachycardia have been reported. There have been rare reports of QT prolongation and torsades de pointes.

Skin and Subcutaneous Tissue Disorders:

Frequency not known: severe cutaneous adverse reactions (SCARs) including Stevens Johnson syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP).

Postmarketing Experience:

Cardiac Disorders: Palpitations and arrhythmias including ventricular tachycardia have been reported. There have been rare reports of QT prolongation and torsades de pointes (see Warnings and Precautions).

Gastrointestinal Disorders: infantile hypertrophic pyloric stenosis.

DRUG INTERACTIONS

- Concurrent use with aluminium and magnesium containing antacids will decrease the peak serum concentration in studies conducted; azithromycin should be administered at least 1 hour before or 2 hours after antacids.
- Concurrent use with theophylline has been associated with increased in the serum concentrations of theophylline.
- Concurrent use with warfarin has been associated with increased anticoagulant effects.
- Concurrent use with macrolide antibiotics has been associated with increased serum concentrations of digoxin, carbamazepine, terfenadine, cyclosporine, hexobarbital and phenytoin.
- Concurrent use with ergotamine or dihydroergotamine has been associated with acute ergot toxicity characterized by severe peripheral vasospasm and dysesthesia.

- Concurrent use with triazolam has been associated with a decrease in the clearance of triazolam, which may increase its effects.

OVERDOSE

Adverse events experienced in higher than recommended doses were similar to those seen at normal doses. In the event of overdosage, general symptomatic and supportive measures are indicated as required.

DOSAGE AND ADMINISTRATION

Azimax-250 Tablet should be given as a single daily dose. It can be taken with or without food.

Children over 45 kg body weight and Adults, including elderly patients:

The total dose of azithromycin is 1500 mg which should be given over three days (500 mg once daily). In uncomplicated genital infections due to Chlamydia trachomatis, the dose is 1000 mg as a single oral dose.

In children under 45 kg body weight:

Azimax-250 Tablet is not suitable for children under 45 kg.

Renal Impairment:

No dose adjustment is necessary in patients with mild to moderate renal impairment (GFR 10 - 80 ml/min). Caution should be exercised when azithromycin is administered to patients with severe renal impairment (GFR < 10 ml/min).

Hepatic Impairment:

Since azithromycin is metabolised in the liver and excreted in the bile, the drug should not be given to patients suffering from severe liver disease. No studies have been conducted regarding treatment of such patients with azithromycin.

Note: The information given here is limited. For the further information, consult your doctor or pharmacist.

Storage:

Store below 30°C. Protect from moisture.

Presentation/Packing:

Blister pack of 1 x 6's, 5 x 6's, 10 x 6's, 3 x 10's, 10 x 10's.

Product Registration Holder: HOVID Bhd.

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Manufactured by: HOVID Bhd.

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